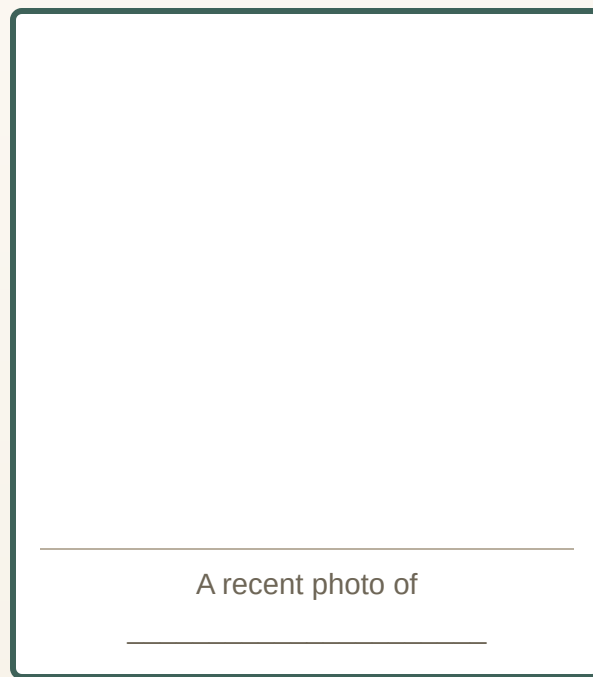




THE DEMENTIA HOUSE CALL

The Dementia House Call Care Binder



This photo goes on the cover for a reason: it reminds everyone who opens this binder — family, paramedics, nurses, doctors — that this person is the centre of every decision.

Everything the care team needs to know — in one place, in your hands.

A practical, fill-in binder for families caring for someone living with dementia: medication lists, appointment tools, a hospital transfer sheet, behaviour logs, and gentle guides for the hard conversations.

Created by a Canadian physician who makes dementia house calls.

Educational information for caregivers — not medical advice.



How to Use This Binder

If you're reading this, you are probably the person everyone else calls "the one who handles things." Maybe you didn't choose the job. Most people don't. You're doing something hard, and the fact that you're organizing information like this means you're doing it well — even on the days it doesn't feel that way.

Here's something I've learned from years of caring for people with dementia: **the family member with a binder changes the appointment.** When you can hand a doctor a current medication list, a one-page summary of what's normal for your person, and three dated notes about what's changed — you stop being "the worried family member" in the room and become a colleague on the care team. Care gets better. Visits get shorter. Fewer things fall through the cracks.

THIS BINDER IS A LIVING DOCUMENT

One principle before anything else: **this binder is a living document, and no filled page should ever be thrown away.** Pages will fill up, lists will change, entries will be crossed out — that's the binder working. Keep the old pages behind the new ones. Together they are this person's **story**, and every part of that story is vital to the care team: a medication stopped two years ago, a behaviour pattern from last spring, the words they used when they could still say what mattered. Never discard a filled page. Add, update, and keep going.

WHAT'S INSIDE

- **Our Team** — one page with every name and number you need
- **Medications** — a master list plus questions to ask at every review
- **When to Call, When to Go** — a traffic-light guide for worrying moments
- **The Hospital Transfer Sheet** — the one page you hand the emergency department
- **Behaviour & Symptom Log** — because behaviour is communication
- **Appointment Prep** — get more out of every visit
- **Goals of Care** — a gentle guide to the conversation that matters most
- **Legal & Planning Checklists** — separate pages for Canada and the United States
- **Your Own Oxygen Mask** — a page that is only about you

THREE SUGGESTIONS

1. **Fill in the "Our Team" page and the medication list first.** In an emergency, those two pages do most of the work.
2. **Use pencil, or print extras.** Things change. That's normal. A messy, current binder beats a tidy, outdated one — and remember, filled pages stay in the binder. They're part of the story.
3. **Keep it where the ambulance would find it.** Many families keep the binder near the fridge or by the front door, and tell home care workers where it lives.

One more thing: nothing in this binder is medical advice for your specific person. It's a tool to help you work *with* their own doctor — the person who knows their history and can advise on their care.

Educational information for caregivers — not medical advice. For decisions about an individual's care, consult their own physician.



THE DEMENTIA HOUSE CALL

Our Team

This binder belongs to (person receiving care):

Preferred name:

Date of birth:

Health card / insurance no.:

Primary language:

FAMILY PHYSICIAN / PRIMARY CARE PROVIDER

Name:

Clinic:

Phone:

Fax:

After-hours line:

SPECIALIST(S) (GERIATRICIAN, NEUROLOGIST, GERIATRIC PSYCHIATRY, MEMORY CLINIC)

Name / role:

Phone:

Name / role:

Phone:

PHARMACY & PHARMACIST

Pharmacy name:

Pharmacist we usually speak with:

Phone:

Delivers? ☐ Yes ☐ No Blister packs? ☐ Yes ☐ No

HOME CARE / CASE MANAGER

Agency:

Coordinator:

Phone:

Days/times workers come:

SUBSTITUTE DECISION-MAKER / AGENT / ATTORNEY (HEALTH DECISIONS)

Name:

Relationship:

Phone:

Document type & location:

POWER OF ATTORNEY (FINANCES)

Name:

Phone:

FACILITY / DAY PROGRAM (IF APPLICABLE)

Name:

Unit/room:

Phone:

Charge nurse / key contact:

IN AN EMERGENCY, CALL FIRST

1. Name:

Phone:

2. Name:

Phone:

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Medications

WHY THIS SECTION MAY BE THE MOST VALUABLE PAGE IN THE BINDER

Most people living with dementia take several medications, often started by different prescribers over many years. Nobody is deliberately careless — but lists drift out of date, and in older adults, medication side effects can look exactly like dementia getting worse: more confusion, more falls, less appetite, more sleepiness. An accurate list, reviewed regularly, is one of the most powerful safety tools a family has.

Keep this list current, and bring it — or a photo of it on your phone — to every appointment, every pharmacy visit, and every trip to the hospital.

THIS LIST IS PART OF THEIR STORY — KEEP EVERY VERSION

This binder is a living document. When a medication list fills up or goes out of date, **don't throw it away** — print a fresh copy of the next page, start again, and keep the old list behind it. A medication stopped last year, and why it was stopped, can matter enormously to a new doctor today. Every filled page is part of this person's story, and every part of that story is vital to the care team.

The Master Medication List lives on its own page, on purpose: **print as many copies of that page as you need** for updates and revisions, and file each completed one behind the current list.

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Master Medication List

Last updated:

Updated by:

Pharmacy:

Blister pack / dosette? ☐ Yes ☐ No

ALLERGIES & BAD REACTIONS

Drug / substance	What happened

Also list in the table below: over-the-counter medicines, vitamins, herbal products, sleep aids, pain relievers, eye drops, creams, and "as needed" medications. Doctors need to know about *all* of them — some interact with prescriptions.

When a medication is stopped, don't erase it. Write the stop date in the last column and leave the row in place. This binder is a living document — never discard a filled list. The record of what was tried, and when it stopped, is part of their story, and every part of it is vital to the care team.



THE DEMENTIA HOUSE CALL

Master Medication List — Table

Medication (name as printed on label)	Dose & how taken	Time(s) of day	Prescribed by	What it's for (in plain words)	Started (approx. date)	Stopped (approx. date)

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Master Medication List — Table

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THE DEMENTIA HOUSE CALL

Master Medication List — Table

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"Bring This to Every Appointment"

The Medication Review Sheet

Once or twice a year — and after every hospital stay — it's reasonable to ask the doctor or pharmacist to sit down and go through the whole list. This has a name: a **medication review**. Asking for one is normal, welcome, and something good clinicians appreciate. You are not questioning anyone's competence. You are doing exactly what the care team hopes a family will do.

There's also a word worth knowing: **deprescribing** — the careful, planned reduction or stopping of medications that may no longer be helping, always done *with* the prescriber, never on your own. In older adults, and especially in dementia, "less" is sometimes the safer, kinder prescription. But stopping some medications suddenly can be dangerous, so these are questions to ask, not changes to make at home.

EIGHT PLAIN-LANGUAGE QUESTIONS TO ASK ABOUT ANY MEDICATION

1. **"Is this still needed?"** — What is it treating, and is that condition still active?
2. **"What is this one actually doing for them right now?"** — Is there a benefit we can see or measure, or is it there out of habit?
3. **"What would happen if we stopped it, or lowered the dose?"** — And if we tried, how would we do that safely, and what would we watch for?
4. **"Is this on the Beers list?"** — The Beers Criteria is a well-known list of medications that are often inappropriate or higher-risk in older adults. It's a screening tool, not a verdict — but it's a fair question for any drug on it.
5. **"Could this medication be causing any of what we're seeing?"** — More confusion, drowsiness, falls, constipation, poor appetite, dizziness on standing — could any medication be contributing?
6. **"Is any of these treating a side effect of another one?"** — This is called a *prescribing cascade*, and it's more common than you'd think.
7. **"Given their goals of care, does this still make sense?"** — Some medications are taken today to prevent problems ten years from now. Is that still the right trade for this person, at this stage?
8. **"Can we make the schedule simpler?"** — Fewer times per day, blister packs, or dropping doses that are frequently missed anyway.

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Medication Review — Notes Page

Date of review:

Reviewed with (doctor / nurse practitioner / pharmacist):

MEDICATIONS WE ASKED ABOUT TODAY

Medication	Question we asked (1–8)	What we were told	Change made?
			☐ Continue ☐ Lower ☐ Stop ☐ Watch
			☐ Continue ☐ Lower ☐ Stop ☐ Watch
			☐ Continue ☐ Lower ☐ Stop ☐ Watch
			☐ Continue ☐ Lower ☐ Stop ☐ Watch

IF SOMETHING WAS STOPPED OR CHANGED

What we're watching for at home:

When to call if we're worried:

Follow-up date:

QUESTIONS WE DIDN'T GET TO (BRING NEXT TIME)

A NOTE ON HOSPITAL STAYS

Medication lists change in hospital more than anywhere else. After *any* hospital stay, it's worth asking the family doctor or pharmacist: **"Can we compare the discharge list to the old list, line by line?"** Ask which changes are meant to be permanent and which were only for the hospital stay. This single habit prevents a remarkable number of problems.

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When to Call the Doctor vs. When to Go to Hospital

Families tell me the hardest part isn't the crisis — it's the *not knowing whether this counts as one*. This guide can't replace judgment about your specific person, but it can give you a frame. When in doubt, call — your provincial or local health line, the doctor's office, or 911. **Nobody sensible will ever fault you for checking.**

FIRST, THE SINGLE MOST IMPORTANT SENTENCE IN THIS BINDER:

"Suddenly more confused than usual" is a medical symptom, not a bad day.

A rapid change — over hours to a couple of days — in alertness, confusion, or behaviour has a name: **delirium**. It is different from dementia. Dementia changes slowly, over months and years. Delirium arrives fast, and it usually means something *e/se* is wrong: an infection, a medication effect, dehydration, pain, constipation, or another treatable problem. Delirium is a reason to seek medical care promptly. Because you know this person's "normal" better than anyone, **you are the delirium detector**. Trust what you notice.

DELIRIUM WARNING SIGNS — A CHANGE FROM *THEIR* USUAL, ARRIVING QUICKLY:

- Much more confused, or confused in a *new way*
- Unusually drowsy, hard to wake, or "not really there"
- Or the opposite: suddenly agitated, restless, frightened
- Seeing or hearing things that aren't there, *when that's new for them*
- Attention that comes and goes — good one hour, lost the next
- New trouble with speech, or day–night reversal that appeared abruptly

If you're seeing this picture, this belongs in the **amber or red** zones on the next pages — amber at minimum, red if they are hard to rouse, rapidly worsening, or you cannot reach a clinician promptly.

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The Traffic-Light Guide

● **GREEN — Note it down, mention it at the next visit**

Slow changes over weeks to months. Write them in the log so the visit is useful.

- Gradually needing more help with dressing, bathing, or finding words
- Slowly changing sleep patterns or appetite drifting down over weeks
- Repeating questions more than before; more misplacing of objects
- Less interest in old hobbies; more napping
- A question about a medication that isn't causing a problem right now

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The Traffic-Light Guide

● **AMBER — Call the doctor's office this week (or your health line today)**

New problems over days, or anything that's worrying you but isn't an emergency.

- More confused than usual over the past day or two, but still alert and rousable
- Possible signs of infection: new cough, burning or foul-smelling urine, low-grade fever
- A fall **without** injury — falls deserve a medical look even when nothing hurts
- Signs of pain: grimacing, guarding a limb, new refusal to walk or be touched
- Not eating or drinking much for more than a day; noticeable weight loss
- New or worsening constipation, or new trouble controlling bladder or bowels
- Refusing medications, or you suspect doses are being missed
- New swelling in the legs; a wound that isn't healing
- Behaviour that's new for them — sudden suspicion, agitation, or withdrawal

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● **RED — Emergency now: call 911 or go to the emergency department**

- Hard to wake, unresponsive, or "not really there" — a major sudden change in alertness
- Sudden severe confusion or agitation that came on over hours
- Signs of stroke: face drooping, arm weakness, new slurred or lost speech — **call 911 immediately**
- Chest pain or pressure; new severe trouble breathing
- A fall with a blow to the head — **especially if they take a blood thinner** — or a suspected broken bone
- A first-ever or prolonged seizure
- Choking, or turning blue
- Significant bleeding; severe pain of any kind
- No urine, or nothing to eat *and* drink, for a full day or more

Take the binder with you. The next section is the one page you hand the emergency team.



THE DEMENTIA HOUSE CALL

Hospital Transfer Sheet

HAND THIS PAGE TO THE PARAMEDICS AND THE EMERGENCY TEAM

Name:

Date of birth:

Health card no.:

Dementia diagnosis (type, if known) & year:

Other major conditions:

Allergies:

Current medication list: see attached / in binder pocket ☐ attached ☐ photo on family phone

BASELINE — WHAT IS NORMAL FOR THIS PERSON (VERY IMPORTANT)

Please compare what you see today to this baseline. A change from baseline may be delirium.

- Normally oriented to: ☐ people ☐ place ☐ time ☐ none of these — and that's their usual
- Normally: ☐ walks independently ☐ walks with aid:

☐ wheelchair ☐ bed-based

- Normally: ☐ speaks in sentences ☐ short phrases ☐ few words ☐ mostly non-verbal
- Bladder — usual state:

-
- Bowel — usual state:

-
- Eats/drinks normally by: ☐ self ☐ with help ☐ modified textures:

-
- Hearing aids? ☐ Glasses? ☐ Dentures? ☐ — *please keep these with them; losing them worsens confusion*

HOW TO COMMUNICATE WITH THIS PERSON

Preferred name:

Responds best to: ☐ calm, slow speech ☐ one question at a time ☐ touch on hand ☐ other:
_____ Things that help when frightened:

Things that make it worse:

Comfort items / topics they love:

GOALS OF CARE (SUMMARY — FULL DOCUMENTS LISTED BELOW)

Documented directive / goals-of-care order exists: ☐ Yes ☐ No Location/designation:

In plain words, what matters most to them:

DECISION-MAKER & CONTACTS

Substitute decision-maker / agent (*please call before major decisions*):

Phone:

Second contact:

Phone:

Family doctor:

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Behaviour Is Communication

HOW TO USE THIS LOG

When someone loses the words to say *"I'm in pain," "I'm frightened,"* or *"I need the washroom,"* the message doesn't disappear. It comes out another way — pacing, calling out, refusing care, striking out, withdrawing. In dementia care we've learned to stop asking *"How do we stop this behaviour?"* and start asking *"What is this behaviour telling us?"*

Almost always, it's telling us about an **unmet need**. The most common ones are worth memorizing, because they're the first things to check:

- **Pain** — the most missed message in dementia. People who can't report pain still feel it.
- **Bladder and bowel** — a full bladder, an infection, or constipation can transform behaviour.
- **Hunger, thirst, fatigue** — basic, and easy to overlook in a hard week.
- **Too much going on** — noise, crowds, rushing, being handled without warning.
- **Too little going on** — boredom and loneliness are needs too.
- **Fear and loss of control** — care tasks like bathing can feel, from the inside, like an ambush by a stranger. The resistance is self-protection, not aggression.
- **Something medical brewing** — remember: sudden change = symptom, not personality.

WHY WRITE IT DOWN?

Because patterns are invisible up close and obvious on paper. Three dated entries showing that agitation happens at 4 p.m., before dinner, on days with no nap — that's not a complaint, that's *data*, and it hands the doctor or the care team something they can actually work with. A log also protects you at appointments: instead of *"she's been difficult lately,"* you can say *"here are five episodes, with times, triggers, and what helped."*

HOW TO LOG WITHOUT JUDGMENT

Describe what a camera would have seen. Not *"he was aggressive"* but *"when I reached to remove his sweater without warning, he grabbed my wrist and shouted."* The first version labels him. The second version contains the clue — *without warning* — and the clue is where the solution lives.

And one gentle reminder: a difficult behaviour is not your failure, and it is not their fault. It is a message from someone doing their best with a brain that is changing. You're learning to translate. This log is your dictionary.

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Behaviour & Symptom Log

Date: _____ Time: _____ Logged by: _____

What happened? *(what a camera would have seen — words used, actions, how long it lasted)*

What was happening just before? *(place, people present, noise, care task underway, TV on, visitor leaving, time since last meal or nap)*

Quick needs check *(tick what you know)*

- Last bowel movement:

Any straining/hard stool? ☐

- Bladder: last void

Any odour, dark urine, new accidents? ☐

- Possible pain signs: ☐ grimacing ☐ guarding ☐ rubbing an area ☐ worse with movement
☐ none seen — Where?

- Sleep last night: ☐ usual ☐ poor ☐ up in the night — details:

- Food & fluids today: ☐ usual ☐ less than usual ☐ very little

- Any medication changes in the past 2 weeks? ☐ Yes:

☐ No

What helped (even a little)? *(moving to a quiet room, a snack, music, going along with their reality, stepping away and returning in 10 minutes)*

What didn't help? _____

Follow-up needed? ☐ Watch for pattern ☐ Mention at next visit (green) ☐ Call doctor this week (amber) ☐ Urgent (red)



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Looking Back: Spotting the Pattern

Every week or so, flip back through your log entries with a cup of tea and ask these questions. Patterns usually show up within five to ten entries.

Time of day: Do episodes cluster? (*Late afternoon is common — the care world calls it "sundowning," and earlier naps, snacks, or lighting changes sometimes soften it.*) Pattern I see:

Triggers: Is there a repeating "just before"? (*a care task, a noisy room, hunger, a certain visitor, rushing*) Pattern I see:

Body clues: Any thread of constipation, poor sleep, low intake, or possible pain running through the entries? Pattern I see:

What reliably helps? (*This is gold. Write it here, and copy it onto the Hospital Transfer Sheet.*)

What I want to tell the doctor or care team, in one sentence:

WHEN TO BRING THE LOG TO A PROFESSIONAL

- Episodes are increasing in number or intensity
- Anyone's safety is at risk — theirs, yours, or a care worker's
- The behaviour appeared *suddenly* (remember: sudden change is a symptom — check the traffic-light guide)
- You're being asked to consider a new medication for behaviour — the log helps everyone judge whether it's truly needed, whether unmet needs were addressed first, and later, whether it's working
- Bring the one-week tracker in this section too, if you're using it — a filled tracker is exactly what the care team hopes to see

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One-Week Behaviour Tracker

For a behaviour that keeps happening, one week of brief notes in time blocks shows what a single hard day never can — and because each row is a time of day, patterns like late-afternoon sundowning become visible at a glance. This is a simplified version of the tracking tools we use in clinics: jot a word or two in the block where something happened (what you saw, and the likely trigger if you spotted one), and bring the finished sheet to the appointment.

Day / Date	Early morning (waking–9 am)	Morning (9–12)	Afternoon (12–4)	Evening (4–8)	Night (8 pm–waking)	What helped today	Flag (G/A/R)
Day 1							
Day 2							
Day 3							
Day 4							
Day 5							
Day 6							
Day 7							

Flag key: G = green (note it, mention at next visit) · A = amber (call the doctor's office this week) · R = red (emergency care now) — see the Traffic-Light Guide.

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One-Week Behaviour Tracker

For a behaviour that keeps happening, one week of brief notes in time blocks shows what a single hard day never can — and because each row is a time of day, patterns like late-afternoon sundowning become visible at a glance. This is a simplified version of the tracking tools we use in clinics: jot a word or two in the block where something happened (what you saw, and the likely trigger if you spotted one), and bring the finished sheet to the appointment.

Day / Date	Early morning (waking–9 am)	Morning (9–12)	Afternoon (12–4)	Evening (4–8)	Night (8 pm–waking)	What helped today	Flag (G/A/R)
Day 1							
Day 2							
Day 3							
Day 4							
Day 5							
Day 6							
Day 7							

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Getting the Most from Every Appointment

Medical visits are short. Ten to twenty minutes, sometimes after weeks of waiting. The difference between a visit that changes things and a visit that evaporates is almost always **preparation** — and that's a skill, not a personality trait. This sheet is the skill, written down.

BEFORE the appointment

The three-question rule. Pick your top three concerns and write them below in order. If you only get through one, make sure it's the first one. Say it in the first minute of the visit: *"The main thing I'm worried about today is ____."*

1. _____
2. _____
3. _____

Pack the binder. ☐ Master medication list (updated) ☐ Behaviour log entries since last visit
☐ Any new symptoms noted in the green/amber zones ☐ Weight changes, falls, hospital or ER visits since last time

Logistics that help the person with dementia: book the time of day they're at their best; bring glasses and hearing aids; bring a snack and water for the wait; if possible, bring a second family member — one to be with your person, one to talk with the doctor.

If something is hard to say in front of them (*driving, safety, moving, your own exhaustion*): it's respectful and normal to send a short note to the office ahead of time, or hand the doctor a written note at the start. Doctors are used to this. It protects your person's dignity while still getting the truth into the room.

DURING the appointment

- Lead with your top concern, in one sentence, with dates: *"Since March, three falls."*
- Describe changes from *their* normal — you are the expert on their baseline.
- When something is decided, ask: **"What should we watch for, and when should we come back?"**
- Before leaving, repeat the plan back in your own words: *"So we're going to , **and call if** . Did I get that right?"* This one habit catches most misunderstandings.



Questions to Ask at the Appointment

QUESTIONS WORTH ASKING (CHOOSE WHAT FITS TODAY)

About a new symptom or change:

1. "Could this be a medication effect, an infection, pain, or constipation — rather than the dementia progressing?"
2. "What would tell us this is getting more serious?"

About a new medication or treatment:

1. "What are we hoping it will do, and how soon would we see it?"
2. "What are the most likely side effects in someone with dementia?"
3. "How will we decide whether it's working — and when would we stop it?"

About the road ahead:

1. "What changes should we expect next, so we're not caught off guard?"
2. "Is there anything we should put in place *now* — home care, day programs, a driving assessment, planning documents — while things are stable?"
3. "Who do we call between visits if something changes?"

For you (yes, this belongs in a medical visit):

1. "I'm the main caregiver. What respite or support could we be referred to?"

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THE DEMENTIA HOUSE CALL

AFTER the Appointment — Visit Summary

Date:

Seen by:

What we were told (plain words):

Medication changes: ☐ none ☐ started ☐ stopped ☐ changed Details:

→ Update the Master Medication List today, while it's fresh.

Tests ordered / referrals made:

We should call if:

Next appointment:

Booked? ☐

One thing I want to remember from this visit:

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The Conversation That Matters Most

A VALUES-FIRST GUIDE TO GOALS OF CARE

There is a conversation most families put off — not because they don't care, but because they care so much. It's the conversation about what kind of care your person would want as their dementia advances. I want to gently make the case for having it **soon, and unhurried**, rather than later, in a hospital hallway, at 2 a.m., with a doctor you've just met.

Here is the reassuring truth: **this conversation is not about death. It's about them.** It's about making sure that when decisions have to be made, they're made in *their* voice — not from panic, and not from guilt.

START WITH VALUES, NOT PROCEDURES

Skip the medical machinery at first. Don't open with "Do you want CPR?" — most people can't meaningfully answer that cold, and it's not really the question. Start here instead, with them if they can still take part, or among family who knew them longest:

- **"What does a good day look like for them?"** What has always made life feel worth living — people, food, music, faith, being outdoors, independence?
- **"What have they said over the years?"** Many people have left clues: reactions to a relative's illness, offhand comments like "don't ever let me live like that," or the opposite — "fight for every day."
- **"What would they *not* want?"** Prolonged hospital stays? Being kept alive without awareness of loved ones? Feeding tubes? Pain? Or would they want every possible intervention, every time?
- **"What do they fear more — dying, or the way of dying?"** For many people, comfort, familiar faces, and freedom from pain matter more than length of days. For others, time itself is the treasure. Neither answer is wrong. The only wrong answer is the one that isn't theirs.

IF THEY CAN STILL TAKE PART

Include them, in whatever way they can manage. Even in moderate dementia, many people can express what matters to them, especially in calm moments, in plain words, one question at a time. Their voice — even a partial one — is the anchor for everything that follows. Write down what they say, with the date. Their exact words are precious later.

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Comfort and Intervention: Understanding the Trade-offs

Every medical decision in advanced illness is a trade. Not "care vs. no care" — there is *always* care. The real question is what each option asks of the person, and what it offers them in return. Here is the honest, gentle version of what those trade-offs look like in advanced dementia. Your person's own doctor can tell you how each applies to *them* — this is only the lay of the land.

Hospital transfers. Hospitals are excellent at fixing specific, fixable problems — a broken hip, a serious infection needing intravenous treatment. But for a person with advanced dementia, hospitals are also disorienting, noisy, and a common trigger for delirium. Sometimes the same problem can be treated where they live. So the question is rarely "hospital: yes or no forever," but "for what kinds of problems would a hospital trip serve *their* goals?"

CPR (resuscitation). On television, CPR usually works. In real life, for frail elderly people with advanced dementia, it rarely restores the life the person had before, and it is a physically forceful intervention. Choosing "no CPR" — often documented as a DNR order — does **not** mean "no treatment." It means that when the heart stops naturally, it is allowed to be the end, while every other comfort and care continues. Many families find this the clearest values question of all: *would they want that, at this stage of life?*



Understanding the Trade-offs, continued

Feeding tubes. When swallowing declines in advanced dementia, families are sometimes asked about tube feeding. It feels like the loving choice — food is love. But the evidence in advanced dementia has not shown that tube feeding lengthens life or prevents complications the way people hope, and it takes away one of the last pleasures: taste, and the closeness of being fed by hand. Careful hand-feeding of favourite foods, for comfort, is a recognized and loving alternative. Ask their doctor what's right for your person.

"Comfort-focused care." This means the goal of every intervention becomes how the person *feels*: treating pain, breathlessness, anxiety, and agitation vigorously; skipping tests and treatments that add burden without adding comfort. It is active, skilled, attentive care — often *more* hands-on than hospital care, not less.

The middle ground is real. Goals of care are not a single switch. Many families land somewhere like: *treat infections where she lives, no hospital unless needed for comfort (like a broken bone), no CPR, no feeding tube, everything for comfort always*. Your person's doctor can help translate values into a written plan — and the plan can change as things change.



Plain-Language Glossary

WORDS YOU'LL HEAR (AND WHAT THEY ACTUALLY MEAN)

Goals of care — The overall aim of treatment for this person at this stage: from "prolong life with every available measure" to "comfort above all," with much in between. Written down, it guides every clinician who meets them.

Substitute decision-maker (SDM) — The person legally entitled to make health decisions when your person no longer can. Depending on where you live, this may be called an agent, attorney for personal care, healthcare proxy, or representative. Their job is not to decide what *they* want — it's to speak with the voice of the person: *"Knowing her, she would say..."*

Advance care planning — The whole process on these pages: thinking, talking, writing it down, and telling the right people where the documents are.

Palliative care — Specialized care focused on comfort and quality of life in serious illness.

Palliative does not mean giving up, and it does not mean death is imminent. People can receive palliative care for years, sometimes alongside other treatment. It means adding a team whose entire expertise is comfort. Families consistently wish they'd asked for it sooner.

Hospice — Care (a place, or a program at home) focused entirely on comfort in the last phase of life. A word for a *kind* of care, not a surrender.

DNR / DNAR — "Do not resuscitate": no CPR when the heart stops. All other care continues.

Delirium — Sudden, medical confusion (see the traffic-light section). Mentioned here because preventing and treating it is part of comfort.

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THE DEMENTIA HOUSE CALL

Family Worksheet

What matters most to them (their words if possible, with date):

What they would not want:

Where we've landed for now (*it's allowed to be provisional*):

Who agrees / who still needs the conversation:

Discussed with their doctor on:

Written document? ☐ Yes ☐ Not yet Where it is / when it's planned:

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Legal & Planning Checklist — CANADA

Important: Terminology and rules differ by **province and territory**, and this page is general education only — it is not legal advice. A lawyer or notary in your province, or your provincial government's public guardian/trustee resources, can tell you what applies to your family.

THE TWO BIG JOBS, AND ALBERTA'S NAMES FOR THEM (AS AN EXAMPLE)

Nearly every province separates planning into two streams. Using **Alberta** as the example:

1. **Health & personal decisions** → **Personal Directive**. In Alberta, a *Personal Directive* is the document where a person names an **agent** to make personal and health-care decisions (medical care, where they live, and other personal matters) if they lose capacity, and can write down their wishes.
2. **Money & property decisions** → **Enduring Power of Attorney**. In Alberta, an *Enduring Power of Attorney* names an **attorney** (not necessarily a lawyer — usually a trusted person) to handle finances and property, and "enduring" means it continues (or takes effect) after capacity is lost.

The same jobs have different names elsewhere — for example, Ontario uses Powers of Attorney for Personal Care and for Property; British Columbia uses Representation Agreements and Enduring Powers of Attorney; Quebec uses a protection mandate. The *concepts* travel; the paperwork doesn't. Check your own province.

DOCUMENTS & QUESTIONS TO ASK ABOUT (CHECKLIST)

- ☐ Health/personal decisions document (Personal Directive or your province's equivalent) — signed? Where is it? Does the named agent know?
- ☐ Financial document (Enduring Power of Attorney or equivalent) — signed? Does the bank have a copy or know about it? **A note from practice: get the financial documents in place early — before, or at the same time as, the health-care documents.** Banks can refuse to honour a power of attorney that was signed after a person's capacity came into question, and families usually discover this at the worst possible moment — when bills, care fees, or urgent legal matters need handling. Sorting finances first means the capacity question is settled for money matters, and the family can manage urgent financial and legal concerns without a court process.
- ☐ A will — current? Executor aware?
- ☐ Goals-of-care documentation with their doctor or care facility (some provinces use formal designation forms — ask what applies)
- ☐ If **no documents exist and capacity is now in question** — ask a lawyer or your provincial public guardian/trustee office about the options; don't assume it's too late until someone qualified says so
- ☐ List of accounts, insurance, pensions (Canada Pension Plan, Old Age Security), and where key papers live
- ☐ Disability Tax Credit and caregiver-related tax credits — ask an accountant or the Canada Revenue Agency whether they apply
- ☐ Copies: decision-makers, the doctor, and this binder ☐ each know where the originals are

Timing matters: these documents can only be *made* while a person still has legal capacity to make them. Earlier is easier, calmer, and keeps their voice in it. If you're unsure about capacity, ask — that's a professional's call, not a family guess.

Educational information for caregivers — not medical advice or legal advice. For decisions about an individual's care, consult their own physician.



Legal & Planning Checklist — UNITED STATES

Important: Terminology and rules differ by **state**, and this page is general education only — it is not legal advice. An elder-law attorney in your state, or your state's aging/legal-aid resources, can tell you what applies to your family.

THE TWO BIG JOBS, IN COMMON US TERMS

1. **Health decisions** → **Advance directive & healthcare proxy.** Most states use some combination of a *healthcare proxy / healthcare power of attorney / durable power of attorney for healthcare* (naming the person who decides) and a *living will / advance directive* (writing down wishes). Names and forms vary state by state.
2. **Money & property decisions** → **Durable power of attorney (finances).** "Durable" means it remains valid after the person loses capacity — in this context, that's the point of it.

You may also hear about **POLST/MOLST-type forms** (portable medical orders for seriously ill people, signed with a clinician — availability and names vary by state) and, if no documents were ever made, **guardianship/conservatorship** — a court process an attorney can explain.

DOCUMENTS & QUESTIONS TO ASK ABOUT (CHECKLIST)

- ☐ Healthcare proxy / healthcare power of attorney — signed? Does the named person know and agree?
- ☐ Advance directive / living will — signed? Copy given to their doctor?
- ☐ Durable power of attorney for finances — signed? Does the bank know? **A note from practice: get the financial documents in place early.** Banks can refuse to honour a power of attorney that was signed after a person's capacity came into question, and families usually discover this at the worst possible moment. Sorting finances early lets the family manage urgent bills and legal matters without a court process.
- ☐ A will (and ask an attorney whether a trust is relevant) — current?
- ☐ Ask their clinician whether a POLST-type form is appropriate at this stage
- ☐ If **no documents exist and capacity is now in question** — consult an elder-law attorney about options before assuming it's too late
- ☐ List of accounts, insurance (including Medicare/Medicaid status), pensions, veterans' benefits if applicable, and where key papers live
- ☐ Ask an elder-law attorney or your Area Agency on Aging about benefits and planning programs in your state
- ☐ Copies: decision-makers, the doctor, and this binder ☐ each know where the originals are

Timing matters: these documents can only be *made* while a person still has legal capacity to make them. Earlier is easier, calmer, and keeps their voice in it. If you're unsure about capacity, ask — that's a professional's call, not a family guess.

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Your Own Oxygen Mask

This page is not about them. It's about you — and it has earned its place in a medical binder, because caregiver health is not a soft extra. In my work I have watched devoted caregivers run themselves into illness, and I can tell you plainly: **the care plan collapses when you do.** Looking after yourself is not stolen from your person. It is part of their care plan.

BURNOUT WARNING SIGNS — READ THIS LIST HONESTLY

- Sleeping badly most nights; exhausted even after rest
- Getting sick more often; skipping your own medical and dental appointments
- Irritable or tearful over small things; snapping at the person you're caring for, then drowning in guilt
- Feeling numb, flat, or resentful — then guilty about *that* too
- Withdrawing from friends; no longer doing a single thing that's just yours
- Relying more on alcohol, food, or scrolling to get through the evening
- Thoughts like "I can't do this anymore" or "everyone would be better off" — **please treat that last one as a reason to talk to your own doctor now. You deserve care too, and help exists.**

Three or more of these, most weeks, is not a character flaw. It's a signal — the same way a red zone on the traffic-light page is a signal. It means the current plan needs more support in it.

RESPIRE: WHAT TO ASK ABOUT (YES, ASKING IS ALLOWED)

Respite means a planned break — hours, days, or weeks — while your person is safely cared for. Ask their doctor, home-care coordinator, or local Alzheimer Society / Alzheimer's Association chapter about:

- ☐ **In-home respite** — a worker comes so you can leave (or just sleep)
- ☐ **Adult day programs** — structured, social daytime care; many people genuinely enjoy them once settled
- ☐ **Short-stay / overnight respite beds** in care facilities — for a real break or a family event
- ☐ **Friends and family who say "let me know if you need anything"** — they mean it, and they need a specific job: "*Tuesday, 2 to 5.*" Keep a list of jobs ready.
- ☐ **Caregiver support groups** (in person or online) — the only rooms where nobody needs the backstory explained

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Permission Slips & Places to Call

PERMISSION SLIPS

- You can be a good daughter and still take Sundays off.
- You can love someone completely and still need to be away from them.
- You can accept help before you're desperate. That's not weakness; that's logistics.
- You can grieve someone who is still alive. That grief is real, and it doesn't mean you love them less.
- You can laugh — at the absurd days, even with them — without betraying anything.
- You can say "I can't take this on" to other family members. Coordinating care *is* the work.
- You can choose a care facility one day, if it comes to that, and still be the one who kept them safe. Placement is a change of address for the care, not the love.
- You are allowed to be a person, not only a caregiver.

WHO TO CALL — CANADA

Alzheimer Society of Canada — plain-language information, and local societies across the country offering support groups, education, and the First Link® program that connects families to help near them. Start at alzheimer.ca and use "Find your area's Alzheimer Society" to reach your provincial or local society directly. Also worth knowing: your provincial **health line (8-1-1 in most provinces)** for health questions, and your provincial mental-health or crisis line for you — or **9-8-8** (Canada's suicide crisis helpline, call or text) if you are in crisis.

WHO TO CALL — UNITED STATES

Alzheimer's Association 24/7 Helpline: 1-800-272-3900 — free, confidential, staffed around the clock, every day of the year, for people living with dementia *and* their caregivers: information, local resources, and someone to talk to at 3 a.m. who understands. Website: alz.org. Also worth knowing: your local **Area Agency on Aging** (findable via the federal Eldercare Locator), and **9-8-8** (Suicide & Crisis Lifeline) if you are in crisis.

And one more number that matters: your own family doctor's. Book yourself an appointment. Say the words "I'm a caregiver and I'm running on empty." That sentence starts more help than you'd expect.

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About The Dementia House Call

The Dementia House Call is plain-language dementia education from a physician who makes house calls — created for the family members who do the real, daily work of caregiving. No jargon, no fear-mongering, no miracle promises. Just the practical things a doctor wishes every family knew, explained the way we'd explain them at your kitchen table.

Created by a Canadian physician who makes dementia house calls.

JOIN THE FREE EMAIL LIST

Caregiving changes month to month, and so does what you need to know. The free Dementia House Call email brings short, practical letters — what to watch for, what to ask, and the occasional permission slip — straight to your inbox.

Sign up at: thedementiahousecall.com

PRINTING & PERSONAL USE

This binder is for your personal, non-commercial use. Print as many copies of the fill-in pages as your family needs.

FULL DISCLAIMER

The Dementia House Call Care Binder provides general educational information for caregivers of people living with dementia. It is not medical advice, legal advice, or financial advice, and it is not a substitute for consultation with qualified professionals. Nothing in this binder creates a physician–patient relationship. No content here should be used to diagnose or treat any condition, or to start, stop, or change any medication — decisions about an individual's care, including all medication decisions, must be made with that person's own physician or qualified health-care provider, who knows their history and circumstances. Legal documents and terminology vary by province, territory, and state; consult a lawyer or your local government resources for guidance that applies to you. Organization names, programs, and phone numbers were accurate at the time of writing but may change; please verify before relying on them. If you believe someone is experiencing a medical emergency, call your local emergency number (911 in Canada and the United States) immediately. To the fullest extent permitted by law, the creator and The Dementia House Call disclaim liability for actions taken or not taken based on the contents of this binder.

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